

MedAI Clinical Summary

Patient Interview & SOAP Note Documentation

Date: January 11, 2026 at 08:40 AM

■ SUBJECTIVE

- Patient reports involvement in a motor vehicle accident (MVA) approximately X hours ago. Patient was the driver and describes a rear-end collision.
- Reports immediate onset of neck pain, currently rated 6/10, which radiates into both shoulders.
- Also experiencing a dull headache, rated 4/10, located in the frontal region.
- Denies any loss of consciousness at the time of the accident or since.
- Reports feeling "shaken up" and experiencing some generalized muscle stiffness, particularly in the upper back.
- Denies numbness, tingling, or weakness in any extremities. No visual disturbances, dizziness, or tinnitus reported.
- Able to ambulate independently.

■ OBJECTIVE

- General: Patient appears in mild distress, guarding cervical spine movements. Moves slowly and cautiously.
- Musculoskeletal: Cervical spine with tenderness to palpation over paraspinal musculature bilaterally. Decreased range of motion in flexion, extension, and rotation due to pain and muscle spasm. Bilateral trapezius muscle spasm noted. No obvious deformity, swelling, or ecchymosis in the neck or upper back. Thoracic and lumbar spine range of motion appears full with mild discomfort on extreme movements.
- Neurological: Cranial nerves II-XII grossly intact. Motor strength 5/5 bilaterally in all four extremities. Sensation intact to light touch. Deep tendon reflexes 2+ and symmetric throughout.
- Vital Signs: Unavailable at this time.

■ ASSESSMENT

- Primary Diagnosis:
- Acute cervical strain (Whiplash-Associated Disorder, WAD Grade I)
- Post-traumatic headache
- Myofascial pain syndrome, secondary to trauma
- Differential Diagnosis:
- Concussion
- Cervical spine fracture or subluxation
- Acute ligamentous injury of the cervical spine
- Intervertebral disc herniation

■ PLAN

- Tests:
- Cervical spine X-ray series (AP, lateral, odontoid views) to rule out fracture or significant instability.
- Medications:
- Ibuprofen 600 mg by mouth every 6 hours as needed for pain and inflammation.
- Cyclobenzaprine 5 mg by mouth at bedtime for muscle spasm.
- Patient advice:
- Apply ice packs to affected areas for 20 minutes, 3-4 times daily for the next 48 hours, then transition to moist heat as tolerated.
- Encourage gentle range of motion exercises within pain limits. Avoid strenuous activities, heavy lifting, or prolonged sitting.
- Monitor for red flag symptoms including worsening headache, new onset of numbness/weakness, visual changes, severe dizziness, or bowel/bladder dysfunction.

- Follow-up instructions:
- Return to clinic in 3-5 days for re-evaluation and physical exam.
- Refer to physical therapy for cervical spine mobilization, strengthening, and pain management modalities if symptoms persist or worsen.
- Present to the Emergency Department immediately for any acute neurological changes or significant increase in pain.

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